Case C

Ms. Taylor — 32F, Yoga Instructor, 6 Months Postpartum | Right-sided low back & buttock pain · VAS 6/10 · Worse: rolling in bed, exiting a car, single-leg yoga poses

Target: 70%+ | Musculoskeletal (MSK) Physiotherapy

■ PROBLEM LIST

- SIJ pain right-sided (VAS 6/10)
- Reduced lumbopelvic motor control
- Ligamentous hypermobility (postpartum)
- PSIS tenderness on palpation
- Possible pelvic floor weakness
- Unable to perform single-leg yoga postures
- Difficulty rolling in bed / disrupted sleep
- Pain exiting a car (asymmetric load)
- Unable to teach / demonstrate yoga fully
- Baby care tasks (lifting, feeding positions)

■ RED FLAGS

None identified

■ PATHOPHYSIOLOGY

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■ DIFFERENTIAL DIAGNOSES

1. Lumbar Radiculopathy (L5/S1)

- Low back and buttock pain referral pattern
- Could worsen with sustained postures
- Supporting: buttock pain + occasional leg symptoms in some patients

2. Hip Joint Pathology (FAI / Labral Tear)

- Groin/buttock pain in a flexible, high-demand athlete
- Could be aggravated by yoga postures (extreme ROM)
- Supporting: young active female, demanding hip positions

■ OBJECTIVE ASSESSMENTS & HOW TO PERFORM

Thigh Thrust Test (Posterior Shear Test)

Test 1: Thigh Thrust Test (Posterior Shear Test) What it measures: Posterior SIJ ligament integrity and joint stability under posterior shear force. High sensitivity (88%) for SIJ pain. How to perform: Patient lies supine on the plinth. Flex the test hip to 90° and slightly adduc

Sacral Thrust Test

Test 2: Sacral Thrust Test What it measures: Anterior shear stress across both SIJs simultaneously — tests the integrity of the anterior SIJ ligaments and evaluates bilateral posterior ligament stress. How to perform: Patient lies prone on the plinth, arms by sides. Stand to the

Active Straight Leg Raise (ASLR)

Test 3: Active Straight Leg Raise (ASLR) What it measures: Force-closure capacity of the lumbopelvic region; specifically assesses whether the patient can transfer load through the pelvis efficiently. A key functional test in postpartum PGP research (Mens et al.). How to perform:

Technique: Sacroiliac Joint Mobilisation (Grade I–II Posterior-Anterior Glide) + Soft Tissue Release to Posterior SIJ Ligaments

Technique: Sacroiliac Joint Mobilisation (Grade I–II Posterior-Anterior Glide) + Soft Tissue Release to Posterior SIJ Ligaments Rationale: Low-grade PA mobilisation is indicated to reduce joint irritability and restore afferent input — not to "correct" alignment. I use Grade I–II

■ TREATMENT PLAN

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